

Second-opinion report

Anonymised sample, reproduced with the patient's written consent.

PATIENT

A. A., DOB 12/04/1974

STUDY

MRI brain with contrast, 18/01/2026

REPORTING CONSULTANT

Dr D. Mallon, FRCR, GMC 7012345

COMPARED WITH

MRI brain, 03/2024

CLINICAL QUESTION

Previous report raised a "possible left temporal lesion". Patient asks whether this is a tumour and whether it has changed since 2024.

FINDINGS

The area described in the left temporal region corresponds to a prominent developmental venous anomaly, measuring 8 mm, unchanged from the 2024 study (8 mm). No mass effect, no restricted diffusion and no abnormal enhancement of the adjacent parenchyma.

The remainder of the brain is within normal limits for age. Ventricles and sulci are of normal calibre. No acute infarct, haemorrhage or space-occupying lesion. Major intracranial vessels show normal flow voids.

Paranasal sinuses and mastoid air cells are clear. Orbits unremarkable.

OPINION

Prominent developmental venous anomaly, left temporal lobe. A normal anatomical variant. Stable since 2024. No tumour, and no radiological explanation for the reported headaches.

What this means for you

Written for the patient, not for a clinician.

The thing seen on your scan is a normal variation in the way a small vein has formed. It is not a tumour and it has not grown since 2024. It does not need treatment and it is not the cause of your headaches.

Developmental venous anomalies are common — they are found in roughly one in twenty people having a brain MRI, and most never know they have one. They do not turn into anything else over time, and they do not need to be watched with repeat scans.

That means the answer to your question is a straightforward no. Your headaches will have another explanation, and that is worth pursuing with your GP.

RECOMMENDED NEXT STEPS

1. Share this report with your GP.
2. Ask your GP to review headache causes unrelated to imaging.
3. No further imaging required on radiological grounds.

SIGNED

Dr D. Mallon, FRCR — Consultant Neuroradiologist, GMC 7012345
4 August 2026, 14:02

This report is a radiological opinion based on the images supplied and does not replace clinical assessment by your treating doctor. SecondRead is not an emergency service — if you are unwell, contact 111 or 999.

Sample document. Patient details, dates and the report reference have been changed. The clinical content is reproduced with the patient's written consent.